PROVIDENCE MERCY HOSPITAL
Fall River, Massachusetts

IN-PATIENT CLINICAL RECORD

Name:	Brennan, Arthur Leonard
Unit No.:	22-84-016
Ward:	C - Men's Medical, Bed 11
Age:	51 yrs.
Born:	11 Sept 1919 - New Bedford, Mass.
Soc. Sec.:	327-56-4188
Address:	58 Marlborough Row Fall River, Mass. 02720
Telephone:	OS 4-7719
Occupation:	Loom fixer, Bourne Mills (28 yrs.)
Married:	Yes - wife Mrs. Edith Brennan
Religion:	Roman Catholic - Fr. Cornelius Doyle notified
Admitted:	17 Jan 1971, 11.40 a.m.
Admitting Dr.:	R. W. Ashcroft, M.D.
Referred by:	J. P. Mulcahy, M.D. - Pleasant St. office
Adm. Diagnosis:	Bleeding peptic ulcer? - anaemia
Discharged:	2 Feb 1971
Final Diagnosis:	Chronic duodenal ulcer with haemorrhage; iron deficiency anaemia; chronic bronchitis

Next of kin: Mrs. Edith Brennan (wife), same address.
Insurance: Blue Cross of Mass. No. 41-9927-B
Employer's certificate on file - Bourne Mills, personnel dept.

ADMISSION NOTE

17 Jan 1971 Ward C

This 51 year old married white male loom fixer is admitted at 11.40 a.m. complaining of black tarry stools for 4 days and increasing weakness. Referred from the office of Dr. J. P. Mulcahy who found the haemoglobin 7.2 gm.

Patient states he has had burning epigastric pain for about 3 years, relieved by milk and by soda. Pain wakes him at 2 a.m. most nights. He has been taking soda powders from the chemist and, latterly, aspirin for a bad shoulder - as many as 12 tablets a day.

Four days ago he passed a large black stool, and again the following morning. He felt faint at the loom on Friday and was sent home by the foreman. No frank haematemesis. No vomiting. Appetite poor for a fortnight. Weight down about 10 lb.

Past history: Pneumonia 1943 while in the Army. Appendix out 1951, Union Hospital. Smokes 1 1/2 packs Camels daily since age 17. Beer on Saturdays. No known drug allergy - penicillin given 1943 without trouble.

Family: Father died 62, 'stomach trouble'. Mother living, 78, diabetic. One brother, well. Two children, both well.

R. W. Ashcroft, M.D.

PHYSICAL EXAMINATION

17 Jan 1971

T. 98.6 P. 108 R. 22 B.P. 104/68 lying
86/56 sitting

Wt. 148 lb. Ht. 5 ft. 9 in.

General: Pale, thin man, looks older than stated age.
Alert, co-operative, in no acute distress.

Skin: Pale, cool. No jaundice, no spider naevi.

Head: Normal. Conjunctivae very pale.

Mouth: Poor dentition, several teeth missing.
Tongue smooth and red.

Neck: Supple. No nodes. Thyroid not enlarged.
Neck veins flat.

Chest: Emphysematous. Scattered rhonchi both bases,
clearing with cough. No rales.

Heart: Rapid, regular. Soft systolic murmur at apex,
grade 1/6, thought haemic. No gallop.

Abdomen: Soft. Tenderness in epigastrium without
guarding or rebound. Liver 1 finger below
costal margin, smooth. Spleen not felt.
Old appendix scar, well healed.

Rectal: Black tarry stool on glove. Guaiac ++++.
Prostate 1+, smooth.

Extrem.: No oedema, no clubbing. Nails brittle.

C.N.S.: Cranial nerves intact. Reflexes 2+ and equal.
No pathological reflexes.

Impression: 1. Upper G.I. haemorrhage, probably duodenal
ulcer.
2. Iron deficiency anaemia, severe.
3. Chronic bronchitis.

R. W. Ashcroft, M.D.

PHYSICIAN'S ORDERS

17 Jan 1971 11.55 a.m.

1. Admit Ward C, bed 11. Complete bed rest.
2. Sippy diet, hourly milk & cream 6 a.m. - 10 p.m.
3. Type & cross-match 4 units whole blood.
4. Hgb, Hct, W.B.C. & diff. stat and q. 6 h.
5. B.U.N., electrolytes, prothrombin time stat.
6. Stool for occult blood each specimen.
7. Levine tube - iced saline lavage until clear.
8. I.V. 5% D/W 1000 cc. q. 8 h.
9. Vital signs q. 15 min. x 4, then q. 1 h.
10. Aluminium hydroxide gel 30 cc. q. 1 h. while awake.
11. Phenobarbital gr. 1/2 t.i.d.
12. NO ASPIRIN. NO SALICYLATES OF ANY KIND.
13. Notify H.O. if B.P. systolic under 90.

R. W. Ashcroft, M.D.

17 Jan 1971 6.10 p.m.

14. Transfuse 2 units whole blood now, slowly.
15. Benadryl 50 mg. I.M. before transfusion.
16. Hgb. in a.m.

H. Steinmetz, M.D.

18 Jan 1971 8.30 a.m.

17. Upper G.I. series when bleeding stopped 24 h.
18. Ferrous sulphate gr. 5 t.i.d. p.c. when taking p.o.
19. Continue antacid hourly.
20. Out of bed to commode only.

R. W. Ashcroft, M.D.

NURSES' NOTES

17 Jan 71 12.15 p.m.
Admitted to bed 11 per wheelchair from admitting office. Skin pale and moist. States he feels 'washed out'. Side rails up. Bell within reach. Orientation to ward given to patient and to wife.

Sr. M. Bernadette, R.N.

17 Jan 71 1.30 p.m.
Levine tube passed per Dr. Ashcroft without difficulty. Returns coffee-ground material approx. 200 cc. Iced saline lavage begun. Patient tolerated fairly well.

E. Vasquez, R.N.

17 Jan 71 4.00 p.m.
Lavage now clear. Tube left in place to low suction. B.P. 100/64. Pulse 104. Complains of thirst - mouth care given. Wife at bedside.

E. Vasquez, R.N.

17 Jan 71 7.20 p.m.
First unit of blood started 6.40 p.m., no reaction noted. Temp. 99.0. Patient resting quietly. Fr. Doyle visited.

Miss K. Halloran, R.N.

17 Jan 71 11.00 p.m.
Second unit completed 10.50 p.m. without incident. Slept in short naps. No further tarry stool this shift.

Miss K. Halloran, R.N.

PROGRESS NOTES

18 Jan 1971 Ward C Bed 11

Feels somewhat stronger this morning. No further melaena overnight. Levine tube drainage clear yellow, 340 cc. in 12 hours.

T. 99.2 P. 96 B.P. 112/70

Hgb. 9.4 gm. (was 7.2 on adm.)

Hct. 29% W.B.C. 11,200

Abdomen soft, epigastric tenderness less marked. Bowel sounds active. Chest clear except for occasional rhonchus.

Imp. Bleeding appears to have stopped. Will continue conservative management. Tube may come out this evening if drainage stays clear.

R. W. Ashcroft, M.D.

PROGRESS NOTES

19 Jan 1971 Ward C Bed 11

Levine tube removed 8 p.m. yesterday. Took milk and cream overnight without vomiting. Slept well. Asks for a cigarette - explained why not.

T. 98.8 P. 88 B.P. 118/74

Hgb. 9.8 gm. Hct. 30%

Stool guaiac ++ (was ++++)

Ferrous sulphate begun. Warned patient of black stools from iron so that he is not alarmed, and so that we are not misled.

Upper G.I. series ordered for tomorrow a.m. if he remains stable through tonight.

R. W. Ashcroft, M.D.

PROGRESS NOTES

20 Jan 1971 Ward C Bed 11

Upper G.I. series done this morning by Dr. Okamura. Report on chart: large deformed duodenal cap with a persistent niche on the lesser curve, consistent with chronic ulcer. No gastric lesion. No obstruction.

T. 98.6 P. 84 B.P. 120/76

Hgb. 10.2 gm.

Patient much brighter. Ate soft diet at noon. No pain since admission apart from mild epigastric soreness.

Discussed findings with patient and with Mrs. Brennan. Explained that surgery is not required at this time but that he must give up aspirin altogether and cut down the cigarettes. He agreed to try.

R. W. Ashcroft, M.D.

PROGRESS NOTES
21 Jan 1971 Ward C Bed 11

Quiet night. No pain, no melaena. Bowels moved - stool dark from iron, guaiac now +.

T. 98.4 P. 80 B.P. 122/78

Hgb. 10.6 gm. Hct. 33%

Chest: a few coarse rhonchi at both bases in the morning, clearing after he coughs. Sputum thick and grey. Chest film ordered - query old scarring from the '43 pneumonia.

Ambulating in room. Tolerating six small feedings.

L. Cavanaugh, M.D.

PROGRESS NOTES

22 Jan 1971 Ward C Bed 11

Chest film reported by Dr. Okamura: increased bronchovascular markings, hyperinflation, old fibrotic changes right upper lobe. No active infiltrate. Heart normal size.

T. 98.6 P. 78 B.P. 124/78

Cough productive of grey sputum in the mornings only. This is his usual winter pattern for the past 5 years and is clearly chronic bronchitis of the smoker.

Walked the length of the corridor twice without distress. Appetite good. No epigastric pain.

L. Cavanaugh, M.D.

PROGRESS NOTES

23 Jan 1971 Ward C Bed 11

Uneventful. Complains only of being kept in bed too long and of the hospital food, which I take as a good sign.

T. 98.4 P. 76 B.P. 124/80 Wt. 150 lb.

Hgb. 11.1 gm. Hct. 35% Retic. 4.2%

Good reticulocyte response to iron. Marrow evidently responding well. Continue ferrous sulphate for at least 6 months after the haemoglobin is normal, to fill the stores.

R. W. Ashcroft, M.D.

PROGRESS NOTES

24 Jan 1971 Ward C Bed 11

Seen with Dr. Mulcahy, who came in to see the patient. Agreed on plan for medical management with strict avoidance of salicylates.

T. 98.6 P. 78 B.P. 126/80

Sunday. Family visited, patient in good spirits. Ate all of his dinner. Slept in the chair for two hours in the afternoon.

Plan: gastroenterology opinion tomorrow re. long-term management, then home about the end of the week if all remains well.

L. Cavanaugh, M.D.

PROGRESS NOTES

26 Jan 1971 Ward C Bed 11

Consultation note from Dr. Pemberton on chart. Agrees with medical management; no indication for operation in the absence of obstruction, perforation or continued bleeding.

T. 98.4 P. 76 B.P. 124/78

Hgb. 11.6 gm. Hct. 36%

Stool guaiac negative x 3 specimens. This is the first negative series since admission.

Diet advanced to bland, six feedings. Antacid reduced to 1 and 3 hours after meals and at bedtime.

R. W. Ashcroft, M.D.

Consultation

25 Jan 1971 Requested by Dr. R. W. Ashcroft

Re: Brennan, Arthur L. Unit No. 22-84-016

Thank you for asking me to see this pleasant 51 year old mill worker with a chronic duodenal ulcer which bled on 13-16 Jan.

I have reviewed the films with Dr. Okamura. The cap is grossly deformed and there is a constant niche. The changes are those of long-standing disease. There is no evidence of pyloric obstruction: the stomach emptied well at 6 hours.

The haemorrhage stopped without operation and he has had no further bleeding for 9 days. In my opinion there is no present indication for surgery. Operation would be indicated for recurrent severe haemorrhage, perforation, obstruction, or intractable pain.

Recommendations:

1. Bland diet, six small feedings.
2. Antacid 1 and 3 hours after meals and h.s.
3. Absolute prohibition of aspirin and of the proprietary powders he has been using.
4. Stop smoking, or reduce to under half a pack.
5. Continue iron for 6 months.
6. Repeat upper G.I. in 3 months.
7. Office follow-up with Dr. Mulcahy in 2 weeks.

I shall be glad to see him again if any question arises.

G. Pemberton, M.D.

GRAPHIC AND CLINICAL RECORD

Brennan, Arthur L. Unit No. 22-84-016

Date	T.	P.	R.	B.P.	Wt.
17 Jan	98.6	108	22	104/68	148
18 Jan	99.2	96	20	112/70	--
19 Jan	98.8	88	20	118/74	--
20 Jan	98.6	84	18	120/76	148
21 Jan	98.4	80	18	122/78	--
22 Jan	98.6	78	18	124/78	--
23 Jan	98.4	76	18	124/80	150
24 Jan	98.6	78	18	126/80	--
25 Jan	98.4	76	18	124/78	--
26 Jan	98.4	76	18	124/78	151
27 Jan	98.6	74	18	126/80	--
29 Jan	98.4	74	16	128/80	152
1 Feb	98.6	72	16	126/78	153

Intake and Output

Date	Oral	I.V.	Blood	Urine	Suction
17 Jan	200	2000	1000	750	540
18 Jan	900	1000	0	1150	340
19 Jan	1600	500	0	1400	0
20 Jan	1900	0	0	1650	0
21 Jan	2000	0	0	1800	0

Bowels: 17th - tarry x 1; 18th - none;
19th - dark x 1; 20th - dark x 1;
21st onward - normal colour with iron.

Sputum: grey, mornings, moderate amount.

Sleep: good from 19 Jan, no sedation required after the second night.

LABORATORY REPORTS

Brennan, Arthur L. Unit No. 22-84-016 Ward C

Haematology

Date	Hgb.	Hct.	W.B.C.	Retic.	Plat.
17 Jan	7.2 gm	22%	13,400	1.1%	340,000
18 Jan	9.4	29%	11,200	2.4%	--
19 Jan	9.8	30%	9,800	3.6%	--
21 Jan	10.6	33%	8,400	4.0%	--
23 Jan	11.1	35%	7,900	4.2%	310,000
26 Jan	11.6	36%	7,200	2.8%	--
1 Feb	12.4	38%	6,800	1.6%	--

Differential 17 Jan: polys 74, bands 6, lymphs 16, monos 3, eos 1.

Smear: hypochromic, microcytic. Anisocytosis 2+.

Chemistry

B.U.N. 17 Jan 38 mg% 19 Jan 18 mg%
Sodium 17 Jan 138 mEq. Potassium 3.8 mEq.
Chloride 17 Jan 101 mEq. CO2 26 mEq.
Blood sugar 17 Jan 104 mg%
Serum iron 19 Jan 22 ug% (low)
T.I.B.C. 19 Jan 486 ug% (high)
Total protein 19 Jan 6.2 gm% Albumin 3.6
Prothrombin 17 Jan 88% of control

Serology: V.D.R.L. non-reactive.

Urine: Sp. gr. 1.024, alb. neg., sugar neg., micro. 1-2 W.B.C., no casts.

Stool: Guaiac ++++ (17th), ++ (19th), + (21st), negative x 3 (24-26 Jan).

Blood group: O positive. Cross-match compatible.

A. Chowdhury, M.D., Path.

X-RAY DEPARTMENT REPORT

Brennan, Arthur L. Unit No. 22-84-016
Examination: Upper gastro-intestinal series
Date: 20 Jan 1971 Film No. 71-4418

The oesophagus is normal in calibre and empties freely. The stomach is of average tone with normal rugal pattern. No filling defect and no gastric ulcer crater is demonstrated.

The duodenal cap is markedly deformed and irritable, filling poorly and emptying at once. On the lesser curvature there is a small collection of barium which persists on several films and which I regard as an active ulcer niche, approximately 6 mm.

The remainder of the duodenal sweep is normal. There is no evidence of gastric retention; the stomach was empty at the six hour film.

CONCLUSION: Chronic duodenal ulcer with active niche.
No obstruction. No gastric lesion.

T. Okamura, M.D.

Examination: Chest, P.A. and lateral
Date: 22 Jan 1971 Film No. 71-4502

Hyperinflation with flattening of the diaphragms and increased bronchovascular markings. Old fibrotic and pleural changes at the right apex, unchanged from the film of 1963. No active infiltrate, no effusion. Heart within normal limits.

CONCLUSION: Chronic bronchitis and emphysema.
Old healed disease, right upper lobe.

T. Okamura, M.D.

NURSES' NOTES (continued)

27 Jan 71 7.00 a.m.
Slept through the night. No complaint of pain. Took all of breakfast. Walked to the sun room and back unaided.

Sr. M. Bernadette, R.N.

28 Jan 71 2.00 p.m.
Wife brought in street clothes. Patient anxious to go home. Explained he must wait for the doctor's word. Diet instruction sheet given and gone over with both of them.

E. Vasquez, R.N.

29 Jan 71 9.30 a.m.
Dietitian, Miss Farrow, spent 40 minutes with patient and wife on the six-feeding bland diet. Wife has written out a week of menus. Both appear to understand.

Sr. M. Bernadette, R.N.

31 Jan 71 8.00 p.m.
Quiet day. Bowels moved, normal colour. No pain. Smoked one cigarette in the sun room - reminded of the doctor's advice, says he is down from 30 a day to 8.

Miss K. Halloran, R.N.

2 Feb 71 10.15 a.m.
Discharged to home in the care of his wife. Prescriptions and diet sheet given. Valuables returned from the safe and signed for. Left the ward by wheelchair at 10.40 a.m.

E. Vasquez, R.N.

DISCHARGE SUMMARY
2 Feb 1971

Brennan, Arthur Leonard Unit No. 22-84-016
Admitted 17 Jan 1971. Discharged 2 Feb 1971. 16 days.

FINAL DIAGNOSES:

1. Chronic duodenal ulcer with haemorrhage.
2. Iron deficiency anaemia, secondary.
3. Chronic bronchitis and emphysema.
4. Salicylate ingestion, excessive.

This 51 year old loom fixer entered with a 4 day history of melaena and a haemoglobin of 7.2 gm. He was treated with bed rest, gastric lavage, hourly antacid, and 2 units of whole blood on the day of admission. Bleeding ceased within 24 hours and did not recur.

Upper G.I. series on 20 Jan showed a deformed duodenal cap with an active niche. Dr. Pemberton saw him in consultation on 25 Jan and advised against operation. Stools became guaiac negative on 24 Jan and remained so.

The haemoglobin rose steadily on oral iron to 12.4 gm at discharge with a good reticulocyte response. He gained 5 lb. and was ambulant and free of pain for the last ten days of his stay.

The importance of avoiding aspirin was impressed upon him repeatedly, this being in all likelihood the cause of the haemorrhage.

R. W. Ashcroft, M.D.

DISCHARGE INSTRUCTIONS

Given to patient and wife, 2 Feb 1971

Patient: Mr. Arthur L. Brennan
58 Marlborough Row, Fall River, Mass.
Telephone OS 4-7719

MEDICINES:

Ferrous sulphate gr. 5 -- three times a day after meals. Continue six months. Will darken the stool.

Aluminium hydroxide gel 30 cc. -- one hour and three hours after each meal and at bedtime.

Phenobarbital gr. 1/2 -- at bedtime if restless.

NO ASPIRIN. NO BUFFERIN, ANACIN, ALKA-SELTZER OR ANY POWDER OR TABLET FOR HEADACHE WITHOUT ASKING THE DOCTOR.

For pain use only the tablets Dr. Mulcahy prescribes.

DIET: Bland, six small feedings, per the sheet given.
No fried food, no spices, no alcohol, no coffee.
Milk between meals.

SMOKING: Cut down to under half a pack. Better none.

WORK: May return to Bourne Mills 1 March 1971, light duty for the first two weeks. Certificate given to Mrs. Brennan for the personnel department.

CALL THE DOCTOR AT ONCE for black stools, vomiting of blood or coffee-ground material, severe stomach pain, faintness or weakness.

RETURN: Office of Dr. J. P. Mulcahy, 16 Feb 1971, 2 p.m.
Repeat X-ray about 20 April 1971.

R. W. Ashcroft, M.D.

Received and understood: Edith Brennan (Mrs.)